

70. **Intussusception is MOST common at the age of;**
- A. Two months to 2 years.
 - B. Five months to 9 months.
 - C. Six months to 18 month.
 - D. First six months of life.
 - E. Three years to 5 years.
71. **Which of the following statements is correct with respect to the Glasgow Coma Scale?**
- A. Localisation of pain scores 4 for motor response.
 - B. Confused speech scores 4 for verbal response.
 - C. Assessment of verbal response should take place on the patient's better side.
 - D. The scale is prognostic of speech outcomes after therapy.
 - E. A motor score of 3 is lowest possible for patients in ICU.
72. **Which organ, of the following is not likely to be the content of a groin hernia?**
- A. Colon.
 - B. Caecum.
 - C. Ovary.
 - D. Terminal Ileum.
 - E. Prostate.
73. **Which of the following is not a cause of acquired groin hernia in females;**
- A. Pregnancy.
 - B. Ascites.
 - C. Unobliterated processus vaginalis.
 - D. Constipation.
 - E. Previous groin hernia surgery.
74. **An elective laparotomy procedure where the proposed wound is considered as "clean-contaminated", implies an infection rate of ;**
- A. 1%.
 - B. 2%.
 - C. 9%.
 - D. 15%.
 - E. 30%.
75. **Palpation of the nipple areolar complex in a 36 year old female with a palpable small nodule at the 3 o'clock position results in blood appearing. Mammogram findings are normal. What is the likeliest diagnosis?**
- A. Intraductal papilloma.
 - B. Breast cyst.
 - C. Intraductal carcinoma.
 - D. Carcinoma in situ.
 - E. Fat necrosis.

76. A 26-year-old man, stabbed in the right intercostal space in the mid-clavicular line and presents to the emergency department. On examination, subcutaneous emphysema of the right chest wall, absent breath sounds, and a trachea shifted to the left are noted and the patient is in respiratory distress. What is the most likely serious diagnosis?
- A. Pneumothorax.
 - B. Tension pneumothorax.
 - C. Massive haemothorax.
 - D. Haemo-pneumothorax.
 - E. Chest wall laceration.
77. On suspicion of rhinorrhoea, nasal secretion can be differentiated from CSF by testing which levels in the secretion?
- A. Glucose.
 - B. Sodium.
 - C. Potassium.
 - D. Chloride.
 - E. Urea.
78. In a patient with pneumothorax and circulatory collapse the first action should be;
- A. Administration of oxygen by mask.
 - B. Endotracheal intubation with C-spine protection.
 - C. Needle thoracocentesis.
 - D. Immediate chest x-ray.
 - E. Paracentesis.
79. The major cause of death following road traffic accidents is;
- A. Head Injury.
 - B. Blunt abdominal trauma.
 - C. Chest trauma.
 - D. Fat embolism.
 - E. Long bone fractures.
80. The most common primary thyroid malignancy is;
- A. Follicular.
 - B. Papillary.
 - C. Anaplastic.
 - D. Medullary.
 - E. Lymphoma.
81. Bones, stones, groan and moans relates to;
- A. Hyperthyroidism.
 - B. Hyperparathyroidism.
 - C. Hypoparathyroidism.
 - D. Hypothyroidism.
 - E. Pheochromocytoma.

82. **In an adult patient with severe facial injuries;**
- A. An immediate danger to life is acute blood loss.
 - B. Airway obstruction should be relieved by cryothyroidotomy.
 - C. Cervical spine injury should be considered after securing a definite airway.
 - D. Aspirated blood may cause airway obstruction.
 - E. Transport to the casualty should be in the supine position.
83. **Which parameter is used to assess nutritional status?**
- A. Serum albumin levels.
 - B. Triceps skin fold thickness.
 - C. Hand grip strength.
 - D. Prothrombin time.
 - E. White cell count.
84. **Which is an incorrect technique in respect to performing an abdominal examination;**
- A. Check the patient has an empty bladder.
 - B. Ask the patient to keep the arms above the head.
 - C. Ask the patient to point to any areas of pain before commencing palpation.
 - D. Warm your hands, and stethoscope, before placing them on the patient.
 - E. If necessary, Distract frightened or anxious patient with conversation or questions.
85. **A careful history is necessary in all patients being considered for inguinal hernia repair. Symptoms which deserve investigations and appropriate treatment prior to proceeding with inguinal hernia repair include the following except;**
- A. Chronic cough.
 - B. Urinary hesitancy and straining.
 - C. Change in bowel habits.
 - D. A specific episode of muscular straining with associated discomfort.
 - E. Constipation.
86. **Acute appendicitis is most commonly associated with which of the following signs;**
- A. Temperature above 36 degrees Celsius.
 - B. Frequent loose stools
 - C. Anorexia, migratory abdominal pains and right lower quadrant tenderness.
 - D. White blood cell count greater than 10,000 per cubic millimetres
 - E. Diarrhoea.
87. **Which of the following most often initiate development of acute appendicitis?**
- A. Viral infection.
 - B. Acute gastroenteritis.
 - C. Obstruction of appendiceal lumen.
 - D. A primary clostridial infection.
 - E. Infection with E. Coli.

88. **The diagnosis of acute appendicitis is most difficult to establish in;**
- A. Persons aged 60 and above.
 - B. Women aged 18-35 years.
 - C. Infants younger than 1 year.
 - D. Pregnant women.
 - E. Post-menopausal women.
89. **The best type of x-ray to locate free abdominal air is;**
- A. A postero-anterior view of the chest.
 - B. An erect view of the abdomen.
 - C. Computed tomography, (CT), of the abdomen.
 - D. A lateral decubitus x-ray, right side up.
 - E. Pelvic x-ray.
90. **Non-surgical causes of acute abdominal pain may include the following except;**
- A. Hyperthyroidism.
 - B. Adrenal insufficiency.
 - C. Pneumonia.
 - D. Diabetic ketoacidosis.
 - E. Sick cell crisis.
91. **The following are true of periumbilical visceral pain; except;**
- a. May originate from the small intestine disease.
 - b. May originate from Appendix disease.
 - c. May originate from proximal colon disease.
 - d. May originate from duodenal disease.
 - e. In some cases may move to the right lower quadrant.
92. **All of the following benign conditions are associated with increased rates of gastric cancer except;**
- A. Pernicious anaemia.
 - B. Multiple endocrine neoplasia type 1, (MEN 1).
 - C. Adenomatous polyps.
 - D. Chronic atrophic gastritis.
 - E. Reflux gastritis.
93. **Which of the following concerning the pathology of gastric cancer is true?**
- A. Distal gastric cancers are becoming more common.
 - B. Intestinal type gastric tumour, resembles colon carcinomas, and have a better prognosis than diffuse type.
 - C. Early gastric cancers are confined to the mucosa and are lymph node negative.
 - D. Broder's histologic grading system correlates well with survival: patients with grade 4 tumours have 5 year survival rates around 65%.
 - E. Gastric cancer does not spread through haematogenous route.

94. **Complete mechanical small bowel obstruction can cause dehydration by the following except;**
- A. Interfering with oral water intake
 - B. Decreasing intestinal absorption of water
 - C. Inducing vomiting
 - D. Causing oedema of the intestinal wall
 - E. Decreasing secretion of water into the intestinal lumen
95. **Meckel's diverticulum most commonly presents as;**
- A. Gastrointestinal bleeding
 - B. Obstruction
 - C. Diverticulitis
 - D. Intermittent abdominal pain
 - E. Dysuria
96. **Question 96 to 97. A 45-year-old man with a history of previous right hemicolectomy for colon cancer presents with colicky abdominal pain which has become constant over the last few hours. He has marked abdominal distension and has had only minimal vomiting of a feculent material. His abdomen is diffusely tender. Abdominal X-ray shows multiple fluid levels with dilatation of some loops to greater than 3 cm in diameter. The most likely diagnosis is;**
- A. Proximal small bowel obstruction.
 - B. Distal small bowel obstruction.
 - C. Acute appendicitis.
 - D. Closed -loop small bowel obstruction.
 - E. Sigmoid volvulus.
97. **In the patient above the most likely cause of obstruction is;**
- A. Recurrence of tumour and post-operative adhesions are likely to be responsible for the obstruction.
 - B. Constipation is a possible cause of obstruction.
 - C. A foreign body may be the cause of obstruction.
 - D. Colonic polyps may be the primary cause of obstruction.
 - E. Ulcerative colitis is a possible diagnosis.
98. **On urgent management of a burns patient, which statement is factual?**
- A. You should create a hypothermic environment for the patient.
 - B. The burn area should be wrapped only in sterile dressing.
 - C. Use of antibiotics should be encouraged right from the time of admission.
 - D. Intravenous access using large bore cannula is highly encouraged.
 - E. Use of alkaline solution to neutralise acids can be use in chemical burns.
99. **The costovertebral angle;**
- A. Best examined with patient lying supine.
 - B. Is the site to palpate for renal tenderness.
 - C. formed by the lower border of the 10th rib and the transverse processes of the upper lumbar vertebrae
 - D. The ribs protect the lower renal poles.
 - E. The costovertebral angle forms an obtuse angle.

100. Palpation of the RUQ of abdomen can elicit the following structures except;

- A. Liver.
- B. Gallbladder.
- C. Duodenum.
- D. Lower pole of the left kidney may be felt.
- E. Costal margin.



MASENO UNIVERSITY

UNIVERSITY EXAMINATIONS 2017/2018

**FIFTH YEAR END YEAR EXAMINATION FOR THE DEGREE OF
BACHELOR OF MEDICINE AND BACHELOR OF SURGERY WITH
INFORMATION TECHNOLOGY**

CITY CAMPUS

SUPPLEMENTARY EXAMINATION

MCS 501: OPHTHALMOLOGY

Date: 23rd May, 2018

Time: 2.00 - 5.00pm

INSTRUCTIONS:

- This paper contains One hundred (100) multiple choice questions each with options A – E.
- Choose ONE single best answer by circling the correct option
- Answer ALL Questions
- Write your admission number on ALL pages



13. A 3 month child presents with persistent tearing on one eye since birth. She also has mild discharge. She has no eye redness, no photophobia or any history of trauma. What's the likely diagnosis?

- A. Conjunctivitis.
- B. Ophthalmia neonatorum.
- C. Congenital glaucoma.
- D. Congenital nasolacrimal obstruction.
- E. Congenital cataract.

14. Primary open angle glaucoma, is a generally bilateral, but not always symmetrical disease, characterized by;

- A. An IOP above 21mmHg.
- B. Peripheral visual field loss.
- C. Glaucomatous optic nerve head damage.
- D. Arcuate field defects
- E. All of the above

15. Endophthalmitis is;

- A. Infection in the orbit.
- B. Infection in the vitreous cavity.
- C. Inflammation of the anterior chamber.
- D. Very common after cataract surgery.
- E. Treated by topical antibiotics.

16. In myopia;

- A. Length of eye ball is short.
- B. Corneal radius of curvature is less.
- C. Lens is less spherical.
- D. Image forms in front of the retina when the patient accommodates.
- E. Patient can see far objects clearly when he exerts accommodation.

17. Symptoms of cataract include all of the following except;

- A. Halos.
- B. Decreased vision in low illumination.
- C. Decreased vision in bright light.
- D. Glare.
- E. Sudden loss of vision.

18. Treatment options in POAG may include all except;

- A. Pilocarpine.
- B. Beta blockers.
- C. Prostaglandin analogues.
- D. Carbonic anhydrase inhibitors.
- E. Atropine.

- 19. WHO grading of trachoma includes all except;**
- A. TF follicles.
 - B. TI inflammation.
 - C. TS scarring.
 - D. TT trichiasis.
 - E. TP pannus.
- 20. Amongst the following, the least common cause of sudden loss of vision is;**
- A. Vitreous haemorrhage.
 - B. Optic neuritis.
 - C. Central retinal venous occlusion.
 - D. Central retina artery occlusion.
 - E. Retinal detachment.
- 21. The easiest option for visual correction of mild - moderate myopia is;**
- A. Glasses.
 - B. Soft contact lenses.
 - C. Rigid gas permeable lenses.
 - D. Laser vision correction procedures.
 - E. Kerato-melieusis.
- 22. Complications of contact lenses include all of the following except;**
- A. Allergy.
 - B. Corneal infiltrates.
 - C. Corneal ulcer.
 - D. Permanent loss of vision.
 - E. Corneal pigmentation.
- 23. Pupil in acute anterior uveitis is;**
- A. Miosed and regular with poor reaction.
 - B. Miosed and irregular with poor reaction.
 - C. Dilated and irregular with good reaction.
 - D. Dilated and regular with poor reaction.
 - E. Mid-dilated and oval with poor reaction.
- 24. According to WHO a person is blind when:**
- A. Vision in better eye is less than 2/60 and/or visual field is less than 30 degrees in better eye.
 - B. Vision in better eye is less than 3/60 and/or visual field is less than 30 degrees in better eye.
 - C. Vision in better eye is less than 3/60 and/or visual field is less than 20 degrees in better eye.
 - D. Vision in better eye is less than 3/60 and/or visual field is less than 60 degrees in better eye.
 - E. Vision in better eye is less than 5/60 and/or visual field is less than 30 degrees in better eye.

37. Retinoscopy is done for;

- A. Examination retina.
- B. Examination optic nerve.
- C. Refractive power of eye.
- D. Axial length of eye.
- E. To find out the power of IOL.

38. A known diabetic patient presented with sudden painless loss of vision in left eye. The following examinations on the patient are critical except;

- A. Retinoscopy.
- B. Direct ophthalmoscopy.
- C. Indirect ophthalmoscopy.
- D. Slit lamp examination.
- E. Examination with triple mirror.

39. A young patient presented with Rosette shaped cataract, which is characteristic of;

- A. Senile cataract.
- B. Complicated cataract.
- C. Secondary to diabetes.
- D. Radiation induced cataract.
- E. Traumatic cataract with blunt trauma.

40. A patient presented with photophobia and watering in left eye. Examination with fluorescein staining revealed dendritic ulcer. This is caused by;

- A. Staphylococci.
- B. Fungal.
- C. Herpes simplex virus.
- D. Herpes Zoster virus.
- E. Mycobacterium.

41. Distichiasis is;

- A. Misdirected eye lashes
- B. Accessory row of lashes
- C. Everted lid margin
- D. Inverted lid margin
- E. Drooping of upper lid

42. Trachoma is associated with all below, except;

- A. Trantas dots.
- B. Follicles.
- C. Papillae.
- D. Herbert's pits.
- E. Corneal pannus.

43. Nasolacrimal duct opens into;

- A. Superior meatus.
- B. Inferior meatus.
- C. Middle meatus.
- D. Nasopharynx.
- E. None of above.

44. A 45 year old patient presented with facial palsy. The epiphora in this patient was due to;

- A. Ectropion.
- B. Entropion.
- C. Lagophthalmos.
- D. Lacrimal pump failure.
- E. Hyper secretion of tears.

45. Foreign body sensation can be produced by all except;

- A. Allergic conjunctivitis.
- B. Viral conjunctivitis.
- C. Cataract.
- D. Contact lens wear.
- E. Trichiasis.

46. The following are common features of thyroid ophthalmopathy, except;

- A. Proptosis.
- B. Ptosis.
- C. Lid retraction.
- D. Conjunctival chemosis.
- E. Extra ocular muscle thickening.

47. A patient presented with gross decrease of vision. Torch examination revealed conjunctival congestion and miotic pupil. In addition, the anterior chamber has inflammatory cells. Which is the probable diagnosis?

- A. Anterior uveitis.
- B. Acute congestive glaucoma.
- C. Conjunctivitis.
- D. Scleritis.
- E. Foreign body.

48. A patient presenting with sudden, painless loss of vision and no view of the retinal fundus is possible. Which of the following investigations is helpful in accessing the retina?

- A. Keratometry.
- B. Pachymetry.
- C. B-Scan ultrasound.
- D. A-Scan ultrasound.
- E. Indirect ophthalmoscopy.

- 49. A hypertensive and diabetic patient presented with sudden painless loss of vision. The following are the likely possibilities except;**
- A. CRAO.
 - B. CRVO.
 - C. Vitreous Haemorrhage.
 - D. Retinal detachment.
 - E. Neovascular glaucoma.
- 50. Myopia is a condition which is;**
- A. Farsightedness.
 - B. Corrected with concave lens.
 - C. Corrected with convex lens.
 - D. Image formed behind the retina.
 - E. Eye ball is small.
- 51. Hypermetropia is a condition in which except;**
- A. Axial length of eye ball is small.
 - B. Correction with convex lens.
 - C. Image formed behind the retina.
 - D. Refractive error can be diagnosed by retinoscopy.
 - E. Rays of light converged behind the retina when accommodation reflex is active.
- 52. A patient operated for cataract four months back comes to you with foggy vision. On examination there was posterior capsular thickening, which mode of treatment is most useful;**
- A. Excimer laser.
 - B. YAG-Laser.
 - C. Argon laser.
 - D. Krypton laser.
 - E. Diode laser.
- 53. In diabetic retinopathy, new vessel formation is due to following;**
- A. Retinal haemorrhage.
 - B. Vitreous haemorrhage.
 - C. Retinal oedema.
 - D. Retinal ischaemia.
 - E. Retinal breaks.
- 54. Esotropia is;**
- A. Divergent squint.
 - B. Latent convergent squint.
 - C. Associated with accommodation reflex.
 - D. Associated with accommodation and hypermetropia.
 - E. None of above.

55. Exophoria is;

- A. Latent convergent squint.
- B. Alternate divergent squint.
- C. Associated with accommodation reflex.
- D. Latent divergent squint.
- E. None of above.

56. Which of the following regarding atropine is true?

- A. Increases intraocular pressure.
- B. Used in neovascular glaucoma.
- C. It causes miosis.
- D. Atropine is weak cycloplegic.
- E. All of above.

57. A pathognomonic clinical sign of proliferative diabetic retinopathy is;

- A. Micro aneurysms.
- B. Hard exudates.
- C. Deep retinal haemorrhages.
- D. Neovascularization of retina.
- E. Vitreous haemorrhage.

58. A one year old child presented with watering in left eye and sticky discharge. On examination regurgitation test was positive. What is the diagnosis?

- A. Nasolacrimal duct blockage.
- B. Common canaliculus blockage.
- C. Punctal atresia.
- D. Buphthalmos.
- E. None of above.

59. Regarding probing and syringing test, which statement is true;

- A. Recommended for acute dacryocystitis.
- B. It should be done in children above three years.
- C. Congenital nasolacrimal duct block.
- D. It's the treatment of choice in chronic dacryocystitis.
- E. None of above

60. Optic disc oedema is seen in all except;

- A. Papilloedema.
- B. CRVO.
- C. Open angle glaucoma.
- D. Hypertensive retinopathy.
- E. Papillitis.

73. Patient had a pterygium, excised a one month ago, and starts to see double vision, this may be due to;

- A. Medial rectus weakness.
- B. Lateral rectus paralysis.
- C. Symblepharon formation.
- D. Recurrence.
- E. Idiopathic.

74. These treatments are useful in preventing the recurrence after pterygium excision except;

- A. Topical antibiotics.
- B. Topical corticosteroids.
- C. Beta irradiation.
- D. 5 FU eye drops.
- E. Conjunctival graft.

75. Which of the following is specific for the diagnosis of allergic conjunctivitis?

- A. Eye redness.
- B. Itching.
- C. Foreign body sensation.
- D. Excessive lacrimation.
- E. Mucoïd discharge.

76. A patient suffered acute onset of facial nerve palsy; the first line of treatment is;

- A. Frequent ocular lubrication.
- B. Lateral tarsorrhaphy
- C. Topical corticosteroids.
- D. Levator muscle resection.
- E. Topical antibiotics.

77. The levator palpebrae superioris is inserted into the following structures except;

- A. Skin of upper eye lid.
- B. Upper border of tarsus.
- C. Bulbar conjunctiva.
- D. Medial orbital margin.
- E. Medial palpebral ligament.

78. Rolling in of the lower lid margin can be due to;

- A. Thermal injury of lid skin.
- B. Facial palsy.
- C. Trachoma.
- D. Ophthalmoplegia.
- E. Chemical burns.

79. The most important examination in case of congenital ptosis is;

- A. The state of extra ocular muscles.
- B. Fundus examination.
- C. Amount of levator function.
- D. Pupillary light reflex.
- E. Visual acuity.

80. Sty is an acute suppurative inflammation of;

- A. Meibomian glands.
- B. Accessory lacrimal glands.
- C. Zeiss glands of the lash follicles.
- D. Sebaceous glands.
- E. Lacrimal gland.

81. Which of the following is not true regarding viral corneal ulcers.

- A. It can resolve spontaneously.
- B. It is caused by a new infection.
- C. Can be treated by scrapping.
- D. Oral antivirals are as effective as topical antivirals in treatment.
- E. It can be caused by temporary reduction in immunity.

82. Which of the following is not a feature of vitamin D deficiency;

- A. Night blindness.
- B. Xerophthalmia.
- C. Bitot spots.
- D. Worsening of the condition by measles infection.
- E. None of the above.

83. Which of the following statements is true:

- A. The corneal epithelium is made up of a mono layer of cells.
- B. The bowman's membrane is a condensation of the corneal stroma.
- C. The stroma is the thinnest layer.
- D. The descemets membrane lies on top of the stroma.
- E. The endothelium is made up of 3 cell layers.

84. On the better eye, which of the following visual acuities is the best?

- A. Counting fingers at 5 metres.
- B. Counting fingers at 3 metres.
- C. 6/60 vision.
- D. Hand movement.
- E. PL vision.

85. Which of the following is not a feature of blunt trauma to the eye?

- A. Hyphema.
- B. Retinal haemorrhage.
- C. Rosette cataract.
- D. Corneal perforation.
- E. Ruptured globe.

86. Which of the following medications is not used in chemical burns to the eye?

- A. Atropine drops.
- B. Steroid drops.
- C. Pilocarpine drops.
- D. Artificial tears.
- E. Tetracycline ointment.

87. Dacryoadenitis is;

- A. Inflammation of the lacrimal gland.
- B. Inflammation of the lacrimal sac.
- C. Inflammation of the adenoids.
- D. Inflammation of the canaliculi.
- E. None of the above.

88. A red eye can be caused all of the following except;

- A. Iritis.
- B. Keratitis.
- C. Pterygium.
- D. Angle closure glaucoma.
- E. All of the above.

89. The trochlear nerve supplies;

- A. The lateral rectus muscle.
- B. The medial rectus muscle.
- C. The superior oblique muscle.
- D. The levator palpebrae superioris.
- E. The inferior rectus.

90. HIV infection is not associated with the following;

- A. Herpes zoster ophthalmicus.
- B. Acute retinal necrosis.
- C. Toxocariasis.
- D. Squamous cell carcinoma of the conjunctiva.
- E. CMV retinitis.

91. With regards to orbital cellulitis, which statement is not true?

- A. It commonly presents with fever.
- B. Extraocular motility is free.
- C. It requires admission.
- D. Visual acuity is used to monitor progress.
- E. Pupillary reactions are affected.

92. Which of the following statements is true of viral keratitis

- A. Herpes simplex virus is a RNA virus.
- B. Herpes simplex type 2 virus causes genital infection.
- C. Viral keratitis is usually due to a new infection.
- D. Herpes simplex type 2 virus causes an oral infection.
- E. None of the above.

93. The following are uses of fluorescein dye except;

- A. In assessing corneal ulcers.
- B. Used when measuring intraocular pressure.
- C. Used in retinoscopy.
- D. Used in angiography.
- E. Used in assessing the tear film.

94. Which of the following beta blockers is not used in treatment of glaucoma;

- A. Betaxolol.
- B. Timolol.
- C. Metipranolol.
- D. Levobunolol.
- E. Propanolol.

95. Which of the following conditions does not have ocular features?

- A. Tuberculosis.
- B. Typhoid fever.
- C. Syphilis.
- D. Ulcerative colitis.
- E. Hypertension.

96. A 60 year old diabetic lady presents with gradual onset bilateral blurring of vision. Her intraocular pressure is 20 mmHg on the right eye and 18mmHg on the left eye. She has an RAPD on the right eye. You also note neovascularisation on the iris. What is the diagnosis?

- A. Non proliferative diabetic retinopathy.
- B. Primary open angle glaucoma.
- C. Proliferative diabetic retinopathy.
- D. Angle closure glaucoma.
- E. Uveitic glaucoma.

97. What's the management of the above condition?

- A. Immediate admission.
- B. Laser photocoagulation.
- C. Topical steroids.
- D. Topical antibiotics.
- E. Topical atropine.

98. Fungal corneal ulcers present:

- A. Acutely after trauma.
- B. Sharply delineated edges.
- C. Severe pain.
- D. Ill-defined feathery edges.
- E. No hypopyon.

99. Which of the following statements is true?

- A. Tropicamide produces more of cycloplegia than mydriasis.
- B. Atropine produces more of mydriasis than cycloplegia.
- C. Pilocarpine can be used to counteract the effect of atropine.
- D. Tetracaine is not used to anaesthetise the cornea.
- E. Homatropine is safer to use than atropine in children.

100. Which cup disc ratio is very suggestive of glaucoma?

- A. 0.1.
- B. 0.2.
- C. 0.4.
- D. 0.6.
- E. 0.3.



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**FIFTH YEAR END YEAR EXAMINATION FOR THE DEGREE OF
BACHELOR OF MEDICINE AND BACHELOR OF SURGERY WITH
INFORMATION TECHNOLOGY**

CITY CAMPUS

SUPPLEMENTARY EXAMINATION

MCS 502: DENTAL HEALTH

Date: 23rd May, 2018

Time: 9.00 - 12.00pm

INSTRUCTIONS:

- This paper contains One hundred (100) multiple choice questions each with options A – E.
- Choose ONE single best answer by circling the correct option
- Answer ALL Questions
- Write your admission number on ALL pages



1. Which **IS NOT** a characteristic of **REVERSIBLE PULPITIS**?
 - A. Sharp, transient pain.
 - B. History of pain days, weeks or months, but generally not getting worse.
 - C. Dull, constant ache
 - D. Sensitive to cold, duration of more than 30 seconds.
 - E. No obvious radiographic changes.

2. Which statement is **FALSE** regarding dental caries?
 - A. The most common chronic infectious disease of childhood.
 - B. It is a pH-mediated disease.
 - C. Glucose is the main aetiological factor.
 - D. It is a multifactorial disease.
 - E. It is an infectious disease.

3. The following statements are **TRUE** about afferent C fibres **EXCEPT**:
 - A. Transmit burning, aching, throbbing pain.
 - B. Are sometimes stimulated by heat.
 - C. Remain excitable even in necrotic tissue.
 - D. Low threshold for stimulation.
 - E. Have a high threshold of stimulation.

4. The lymphatic drainage to the tip of the tongue is to the;
 - A. Left and right Sub mental lymph nodes.
 - B. Left submandibular lymph node.
 - C. Right and left submandibular lymph nodes.
 - D. Right submandibular lymph nodes.
 - E. Inferior cervical lymph nodes.

5. A 38-year-old patient complained of acute dental pain. The attending dentist found penetrating dental caries (dental decay) affecting 21. Which nerve would the dentist need to anaesthetise to work on that tooth?
 - A. Posterior superior alveolar and greater palatine nerve.
 - B. Middle superior alveolar and mental nerve.
 - C. Middle superior alveolar and Nasopalatine nerve.
 - D. Anterior superior alveolar and greater palatine nerve.
 - E. Anterior superior alveolar and Nasopalatine nerve.

6. Which of the following is a risk factor for dental trauma?
 - A. Over jet of 2mm.
 - B. Over jet of 10mm.
 - C. Competent lips.
 - D. Dental caries.
 - E. Posterior Cross bite.

7. Which of the following is the proper management of an avulsed permanent tooth?
- A. The tooth should be re-implanted within 2 hours.
 - B. The root should be thoroughly cleaned using a soft brush.
 - C. The patient should visit the dentist after resting for a few a days.
 - D. The tooth should be splinted for 6 weeks.
 - E. Root canal treatment is initiated after non-vital discolouration.
8. The deciduous teeth lack this type of tooth;
- A. Central Incisor.
 - B. Canine.
 - C. Premolar.
 - D. Molar.
 - E. Lateral incisors.
9. The pterygomandibular raphe is an essentially vertical line of connective tissue indicating the plane of interconnection of the buccinator and the;
- A. Palatoglossal muscle.
 - B. Palatopharyngeal muscle.
 - C. Superior pharyngeal constrictor muscle.
 - D. Middle pharyngeal constrictor muscle.
 - E. Inferior pharyngeal constrictor muscle.
10. At age 7-8 years, which teeth are most likely to be present?
- A. 71, 81, 32, 42.
 - B. 31, 32, 33, 34.
 - C. 31, 41, 36, 46.
 - D. 31, 41, 36, 37.
 - E. 71, 81, 85, 33.
11. An 8-year-old boy presents with a creamish-white swelling in the gingival region buccal to an occlusaly carious 85. What may be the possible diagnosis?
- A. Pulp polyp.
 - B. Gingival warts.
 - C. Lateral periodontal abscess.
 - D. Pulpal granuloma.
 - E. Dento-alveolar abscess.
12. Incipient lesions can be:
- A. Remineralised and are reversible.
 - B. Remineralised but non-reversible.
 - C. Cannot be remineralised but reversible.
 - D. Cannot be remineralised, non-reversible.
 - E. Requires minimal cavity preparation and restoration with Glass Ionomer Cement.

13. All the statements below regarding Tell-Show-Do are **TRUE EXCEPT**;
- A. It is a therapeutic technique that reduces an anxiety evoking stimulus.
 - B. Verbal explanation of procedures must be given using age appropriate phrases.
 - C. Is effective in children below 3 years of age.
 - D. It familiarises the patient with dental setting.
 - E. Technique is not useful when child is crying.
14. The basic fear of a child below 2 years on his first dental visit is related to;
- A. Anxiety from being separated from parents.
 - B. Fear of infection.
 - C. Fear of bur sound.
 - D. Fear of white dress worn by the dentist and nurse.
 - E. Fear of noises from air compressor.
15. Which is an **INDICATION** for use of general anaesthesia or sedation in dentistry?
- A. Uncooperative child with minimal dental needs.
 - B. Parental objection to general anaesthesia.
 - C. Financial, language or cultural barriers.
 - D. Uncooperative child with extensive dental needs.
 - E. Inadequate training to provide safe care.
16. The first permanent molar erupts at;
- A. 11-12 years.
 - B. 7-9 years.
 - C. 5-6 years.
 - D. 2-5 years.
 - E. 16-21 years.
17. A patient complains of loss of taste, and a numb sensation in the left half of his tongue, after removal of his left lower wisdom tooth. Which of the following nerves is most likely to have been injured during removal of this tooth?
- A. Facial nerve.
 - B. Glossopharyngeal nerve.
 - C. Inferior alveolar nerve.
 - D. Lingual nerve.
 - E. Mandibular nerve.
18. The last primary tooth to be replaced by a permanent tooth is usually?
- A. Maxillary canine.
 - B. Mandibular canine.
 - C. Maxillary first premolar.
 - D. Mandibular first premolar.
 - E. Mandibular third molar.

19. Which of the following is generally the longest root of maxillary permanent first molar?
- A. Distal.
 - B. Disto-buccal.
 - C. Mesial.
 - D. Mesio-buccal.
 - E. Palatal.
20. The following complication may result from injury to auriculotemporal nerve during removal of parotid tumour;
- A. Facial palsy.
 - B. Gustatory sweating.
 - C. Orolingual paraesthesia.
 - D. Anorexia.
 - E. Deafness.
21. If patient comes with history of cardiac problem and is taking aspirin, what will be the possible complication if you do extraction;
- A. Increases bleeding time.
 - B. Increases prothrombin time.
 - C. Increases clotting time.
 - D. Increases partial thrombin time.
 - E. Decreases bleeding time.
22. If patient is taking heparin and we have to do surgery immediately, the effects of heparin can be reversed by;
- A. Vitamin K.
 - B. Protamine sulphate.
 - C. Fresh frozen plasma.
 - D. Hydrocortisone.
 - E. 5% dextrose.
23. Vasoconstrictor in local anaesthetics, (LA), solution;
- A. Reduces toxic effects of LA solution.
 - B. Decreases depth and duration of LA.
 - C. Increases bleeding.
 - D. Increases toxic effects of LA.
 - E. Has no effect on efficacy of LA solution.
24. Reducing agent in local anaesthetic solution is;
- A. Methyl paraben.
 - B. Sodium metabisulphite.
 - C. Thymol.
 - D. Adrenaline.
 - E. Ringer lactate.

- 25. Which are the two nerves blocked when injection is given in pterygomandibular space?**
- A. Buccal and lingual nerve.
 - B. Lingual and inferior alveolar nerve.
 - C. Buccal and inferior alveolar nerve.
 - D. Buccal nerve and infra orbital nerve.
 - E. Inferior alveolar and posterior superior alveolar nerve.
- 26. Most common complication of rheumatoid arthritis involving TMJ is;**
- A. Ankylosis.
 - B. Subluxation.
 - C. Osteoma of condyle.
 - D. Recurrent dislocation.
 - E. TMJ pain dysfunction syndrome.
- 27. In case of multiple extractions, teeth that should be removed last are;**
- A. First molar and canine.
 - B. Second molar and canine.
 - C. First and second molar.
 - D. Second molar and first premolar.
 - E. First molar and first premolar.
- 28. Trismus associated with infection of lateral pharyngeal space is related to irritation of the;**
- A. Buccinator.
 - B. Masseter.
 - C. Temporalis.
 - D. Medial pterygoid.
 - E. Lateral pterygoid.
- 29. Death in Ludwig's Angina occurs due to;**
- A. Sepsis.
 - B. Respiratory obstruction.
 - C. Cavernous sinus thrombosis.
 - D. Carotid blow-out.
 - E. Necrotising fasciitis.
- 30. The roof of pterygomandibular space is formed by;**
- A. Temporalis muscle.
 - B. Medial pterygoid muscle.
 - C. Cranial base.
 - D. Lateral pterygoid muscle.
 - E. Greater wing of sphenoid.

31. The distinguishing feature of masticatory space infection is;
A. Pain.
B. Dysphagia.
C. Trismus.
D. Swelling.
E. Paraesthesia.
32. A mandibular dental infection which exits the buccal cortical plate above the muscle attachment will cause abscess of;
A. Buccal space.
B. Masseter space.
C. Vestibular sulcus.
D. Masticatory space.
E. Sublingual space.
33. Which of the following IS NOT a primary mandibular space?
A. Buccal space.
B. Submandibular space.
C. Sublingual space.
D. Canine space.
E. Pterygomandibular space.
34. Odontogenic infections are most commonly caused by;
A. Mixed bacteria.
B. Anaerobic bacteria.
C. Aerobic bacteria.
D. Streptococci.
E. Actinobacillus actinomycetum comitans.
35. In oral and maxillofacial surgery, 'danger space' is known as;
A. carotid sheath.
B. Posterior to carotid sheath in posterior triangle of neck.
C. Posterior to transverse process of vertebrae.
D. Space between alar and prevertebral fascia.
E. Canine space.
36. Which of the following conditions is more commonly known as a 'dry socket'?
A. Acute pulpitis.
B. Periapical periodontitis.
C. Alveolar osteitis.
D. Pericoronitis.
E. Acute necrotizing ulcerative gingivitis.

37. The following contribute to mouth opening,
- A. Medial pterygoid muscle.
 - B. Lateral pterygoid muscle.
 - C. Masseter muscle.
 - D. Temporalis muscle.
 - E. Stylomandibular ligament.
38. TMJ is a;
- A. Ball and socket joint.
 - B. Hinge and axis joint.
 - C. Synovial joint.
 - D. Diarthrodial joint.
 - E. Hinge joint.
39. The zygomatic bone DOES NOT articulate with:
- A. Frontal bone.
 - B. Maxilla.
 - C. Nasal bone.
 - D. Temporal bone.
 - E. Sphenoid bone.
40. In dislocation of the jaw, displacement of the condylar head beyond the articular tubercle of the temporomandibular joint results from spasm of the following muscles;
- A. Buccinator.
 - B. Lateral pterygoid.
 - C. Masseter.
 - D. Temporalis.
 - E. Medial pterygoid.
41. The most common TMJ dislocation is;
- A. Posterior dislocation.
 - B. Anterior dislocation.
 - C. Medial dislocation.
 - D. Postero-medial dislocation.
 - E. Antero-medial dislocation.
42. All are contents of the pterygo-palatine fossa EXCEPT;
- A. Pterygopalatine ganglion.
 - B. Maxillary division of trigeminal nerve.
 - C. Second part of maxillary artery.
 - D. Third part of maxillary artery.
 - E. Nerve to the pterygoid canal.

43. Which nerve is topographically in the closest connection with the parotid gland?
- A. Maxillary nerve.
 - B. Facial nerve.
 - C. Glossopharyngeal nerve.
 - D. Inferior alveolar nerve.
 - E. Marginal mandibular nerve.
44. According to the American Heart Association recommendations, which of the following combinations require antibiotic prophylaxis for invasive dental treatment?
- 1. Organic heart murmur with regurgitation.
 - 2. Cardiac pacemakers.
 - 3. Isolated secundum atrial septal defect.
 - 4. Previous history of endocarditis.
 - 5. Most artificial joint patients.
- A. 1, 4.
 - B. 2, 5.
 - C. 1, 3, 4.
 - D. 1, 4, 5.
 - E. 1,3,4,5.
45. A patient presents with fractures of the maxillae, orbits and ethmoid bones. This midface trauma can be classified as;
- A. Pyramidal fracture.
 - B. Horizontal fracture.
 - C. Le Forte 1.
 - D. Le Forte 2.
 - E. Le Forte 3.
46. A patient is taking Coumadin (Warfarin). Which blood study do you use to determine his status for surgery?
- A. WBC and differentials.
 - B. Platelet count.
 - C. PT.
 - D. PTT.
 - E. INR.
47. While extracting a mandibular third molar, it is noted that the tip of the distal root is missing. The root tip is most likely in the;
- A. Submental space.
 - B. Parapharyngeal space.
 - C. submandibular space.
 - D. Pterygomandibular space.
 - E. Inferior alveolar canal.

48. **All of the following are found in the cavernous sinus EXCEPT:**
- A. Optic nerve.
 - B. Oculomotor nerve.
 - C. Ophthalmic branch of trigeminal.
 - D. Abducens nerve.
 - E. Internal Carotid Artery.
49. **Paraesthesia following fractures is most commonly seen in;**
- A. Fractured neck of the condyle.
 - B. Fractured symphysis of the mandible.
 - C. Zygomaticomaxillary complex fracture.
 - D. Displaced fracture of the coronoid process.
 - E. Fractures at angle of the mandible
50. **Cutting the motor root of the Trigeminal nerve, (V) would affect which of the following:**
- A. Muscles of mastication, tensor veli palatini and levator veli palatine.
 - B. Tensor tympani, tensor veli palatini and levator veli palatine.
 - C. Muscles of mastication, anterior belly of digastric, tensor tympani and tensor veli palatine.
 - D. Muscles of mastication, posterior belly of the digastric and anterior belly of the digastric.
 - E. Muscles of mastication, extrinsic muscles of the tongue and tensor tympani.
51. **Which of the following best describes the retrodiscal tissues referred to as the bilaminar zone of the TMJ?**
- A. Elastic and fibrous tissue that is highly innervated by the auriculotemporal nerve.
 - B. A thin ligamentous structure that is reinforced laterally by the temporomandibular ligament.
 - C. Avascular hyaline cartilage.
 - D. Vascular, highly innervated fibrocartilage.
 - E. Fibrous connective tissue innervated by the temporal nerve.
52. **A small accidental opening into the maxillary sinus results from removal of a maxillary first molar, but the sinus membrane shows no perforation. The preferred initial treatment is to;**
- A. Elevate a buccal sliding flap and cover the entire extraction site.
 - B. Rotate a pedicle flap from the palate over the socket and suture tightly with non reabsorbable suture.
 - C. Have the patient perform the valsalva maneuver to see if bubbles can be seen in the socket.
 - D. Pack the socket with antibiotic-saturated iodoform gauze.
 - E. Perform routine postoperative care to ensure the formation of, and organisation of a clot.

53. A patient with a full complement of teeth presents with a non-displaced fracture through the body of the mandible. Treatment of choice is:
- A. Intramedullary fixation.
 - B. Circumferential wiring.
 - C. Kirshner wire insertion.
 - D. Closed reduction with intermaxillary wiring.
 - E. If favorable, no fixation required.
54. What is the most common indication for removal of third molars?
- A. Damage to adjacent teeth.
 - B. Destruction of bone on second molars.
 - C. Orthodontic considerations.
 - D. Pericoronitis.
 - E. Associated cysts and tumours.
55. Penicillin is still the antibiotic of choice for oral infections, however, other Beta lactams may be the best in all of the following situations EXCEPT;
- A. Development of resistant organisms.
 - B. Cultures confirm that infection is from mixed aerobes and anaerobes.
 - C. Need for increased gram negative organism coverage.
 - D. Need for greater activity against anaerobes.
 - E. Patient is immunocompromised.
56. A patient has sustained bilateral fractures in the region of the mandibular condylar necks. There are no other injuries. Clinical signs will include;
- A. Inability to protrude the mandible.
 - B. Inability to contact the molar teeth.
 - C. Anterior closed bite.
 - D. Gross unilateral deviation of the mandible on excursion attempts.
 - E. Mental nerve anaesthesia.
57. The following statements are TRUE of sickle cell anaemia EXCEPT;
- A. Radiographs may show a stepladder trabecular pattern.
 - B. Greater than 50% Nitrous oxide use in the treatment of patients is contraindicated.
 - C. A crisis may be precipitated by odontogenic infections.
 - D. Prophylactic antibiotic coverage is indicated for routine procedures.
 - E. Delayed eruption patterns are often seen.
58. The vast majority of salivary calculi are found in the duct of which of the following glands?
- A. Sublingual.
 - B. Parotid.
 - C. Submandibular.
 - D. Minor salivary glands.
 - E. Equally in A and C.

59. The International Normalised Ratio (INR), is a value that would be reported for which of the following laboratory tests;
- A. Platelet Count.
 - B. Prothrombin Time (PT).
 - C. Partial Thromboplastin Time (PTT).
 - D. Bleeding Count.
 - E. None of the above.
60. A definitive diagnosis of oral hairy leucoplakia requires the;
- A. Demonstration of Coxsackie A group virus.
 - B. Demonstration of Epstein-Barr virus-DNA.
 - C. Absence of koilocytes in the spinous cell layer.
 - D. Presence of Fordyce's granules.
 - E. Presence of Oral Herpes virus.
61. A radiopaque periphery encircling a radiolucency is suggestive of a lesion that is;
- A. Slowly growing.
 - B. Rapidly growing.
 - C. Beginning to heal.
 - D. Traumatically induced.
 - E. Recently developed.
62. All of the following are TRUE about the use of Tranexamic Acid in oral surgical procedures EXCEPT;
- A. It is indicated for patients on Coumadin therapy.
 - B. It is indicated for patients with platelet disorders.
 - C. It negates the fibrinolytic activity of saliva following surgery.
 - D. It has a protective effect on the integrity of the clot.
 - E. It is dispensed as a 50 mg solution to be used 4 times a day for 5 days.
63. Chlorhexidine is thought to;
- 1. Alter the cell membrane of bacteria.
 - 2. Be effective due to its substantivity.
 - 3. Be bactericidal by causing precipitation of cytoplasmic contents.
 - 4. Have a pronounced effect on the subgingival microflora.
 - 5. Cause epithelial desquamation.
 - 6. Increase surface staining of composite restorations.
- A. 1, 2, 3, 4, 6.
 - B. 2, 3, 4, 5, 6.
 - C. 1, 2, 3, 5, 6.
 - D. 2, 3, 5, 6.
 - E. All of the above.

64. Several factors predispose diabetics to periodontitis. Which are correct?
1. Elevated glucose levels in oral fluids can influence microbial flora.
 2. Impaired erythrocyte function, including phagocytosis may reduce resistance to periodontitis.
 3. Altered collagen metabolites and vascular changes including stasis.
 4. Impaired chemotactic and phagocytic activity of polymorphonuclear leukocytes.
- A. 1, 2, 3.
B. 1, 3, 4.
C. 2, 3, 4.
D. 3, 4.
E. All of the above.
65. All of the following have shown some clinical correlation with periodontitis EXCEPT;
- A. Cardiovascular disease.
B. Stroke.
C. Pernicious anaemia.
D. Low birth weight babies.
E. Respiratory diseases.
66. Metronidazole, (Flagyl) is;
1. Effective against anaerobes and specific aerobic organisms.
 2. Primarily effective against anaerobes.
 3. Effective in the management of cellulitis and early acute infections.
 4. Should be used in combination with penicillin for serious odontogenic infections.
- A. 1, 4.
B. 2, 3.
C. 2, 4.
D. 1, 3, 4.
E. 2, 3, 4.
67. What is the most common bone deformity found in periodontal disease?
- A. Furca invasion
B. Infrabony pocket
C. suprabony pocket
D. Interdental crater
E. Facial clefts
68. Initial treatment to relieve symptoms of necrotizing ulcerative gingivitis (NUG/NUP) should begin with?
- A. Superficial debridement.
B. An alcohol-based mouthwash and topical antibiotic.
C. Systemic antibiotic therapy.
D. Complete scaling and curettage.
E. Brushing and using chlorhexidine rinses.

69. A 16 year old female attends your dental practice for an appointment. She is fit and healthy but has a phobia of dental injections. During administration of an inferior dental nerve block she becomes unwell and faints. What do you think is the most likely cause?
- A. Addisonian crisis.
 - B. Cerebrovascular accident.
 - C. Hypoglycemic shock.
 - D. Myocardial infarction.
 - E. Vasovagal attack.
70. A 60 year old male with a history of Insulin-dependant Diabetes Mellitus loses consciousness during the course of dental treatment. What do you think is the most likely cause?
- A. Addisonian crisis.
 - B. Anaphylaxis.
 - C. Hyperglycaemia.
 - D. Hypoglycaemia.
 - E. Ketoacidosis.
71. A 4 year old, child presents with fever, gingival bleeding and extensive oral ulceration in his mouth. His medical history is unremarkable. What would be your most likely diagnosis?
- A. Candidal infection.
 - B. Herpes Simplex infection.
 - C. Herpes Zoster infection.
 - D. Paramyxovirus infection.
 - E. Recurrent Aphthous ulceration.
72. A 60 year old female presents with recurrent oral and corneal ulceration. Her biopsy specimen reveals sub-epithelial bullae. What is your most likely diagnosis?
- A. Candidal infection
 - B. Lichen Planus
 - C. Mucous membrane pemphigoid
 - D. Pemphigus vulgaris
 - E. Squamous cell carcinoma
73. A 55 year old female presents with severe, electrical and shocking pain in her right cheek triggered by light touch. Clinical examination does not reveal any local cause. Which drug is most likely to be effective for her?
- A. Baclofen
 - B. Carbamazepine
 - C. Ergotamine
 - D. Paracetamol
 - E. Sumatriptan

74. Which one of the following is the least likely indication for antibiotic prophylaxis to prevent bacterial endocarditis;
- A. Congenital heart disease.
 - B. Diabetes mellitus.
 - C. Previous history of endocarditis.
 - D. Rheumatic fever.
 - E. Valvular heart disease.
75. A 55 year old edentulous female presents with an erythematous patch on her palate. She wears complete dentures and has a history of Insulin Dependent Diabetes Mellitus. You make a diagnosis of Denture stomatitis. Which drug is most likely to be effective for her?
- A. Augmentin.
 - B. Hydrocortisone.
 - C. Ibuprofen.
 - D. Miconazole.
 - E. Phenytoin.
76. A 35 year old female presents with clicking in her right temporomandibular joint for the last six months. She does not have any pain or limitation of her mouth opening. Clinical examination does not reveal Odontogenic cause. What does this condition most likely represent?
- A. Ankylosis of temporomandibular joint.
 - B. Fracture of the mandibular condyle.
 - C. Internal derangement of temporomandibular joint.
 - D. Myofascial pain.
 - E. Rheumatoid arthritis.
77. Which one of the following conditions is least likely to be associated with immunosuppression;
- A. Acromegaly.
 - B. Agammaglobulinemia.
 - C. Anaemia.
 - D. Diabetes mellitus.
 - E. Leukaemia.
78. Bone pain, renal calcinosis, and giant cell jaw lesions are most likely to be seen in which one of the following disorders?
- A. Acromegaly.
 - B. Fibrous dysplasia.
 - C. Hyperparathyroidism.
 - D. Hyperthyroidism.
 - E. Paget's disease.

79. A 45 year old female presents with a 6 month history of recurrent mealtime swelling and pain in her right submandibular region. Clinical examination does not reveal any dental abnormality. What is the most likely cause of her symptoms?
- A. Lymphadenitis.
 - B. Sialolithiasis.
 - C. Sialometaplasia.
 - D. Sialorrhea.
 - E. Xerostomia.
80. A 55 year old male presents with haemorrhagic crusting of his lips following a course of antibiotic therapy. He does not have any systemic manifestations. What would be your most likely diagnosis?
- A. Anaphylaxis.
 - B. Bechet's syndrome.
 - C. Erythema multiforme.
 - D. Lichen Planus.
 - E. Pemphigus vulgaris.
81. Bifid ribs, multiple radiolucent lesions of the jaws, multiple basal cell nevi and falx cerebri calcification are found in;
- A. Basal cell nevus syndrome.
 - B. Sturge weber syndrome.
 - C. Horner syndrome.
 - D. Hereditary internal polyposis.
 - E. Treacher colin syndrome.
82. Ameloblastoma most frequently occurs in;
- A. Mandibular molar region.
 - B. Maxillary molar region.
 - C. Mandibular premolar region.
 - D. Maxillary premolar region.
 - E. Mandibular canine region.
83. Compound odontoma shows on a radiograph as;
- A. Supernumerary teeth.
 - B. Radiolucent and radiopaque areas.
 - C. Masses of calcified areas.
 - D. Distinguishable tooth like structures.
 - E. Starry sky appearance.
84. The most common odontogenic tumour which occurs in relation to an unerupted tooth in the anterior maxilla;
- A. Odontogenic adenomatoid tumour.
 - B. Odontoma.
 - C. Myxoma.
 - D. Cementifying fibroma.
 - E. Odontogenic Keratocyst.

85. One of them **IS NOT** a true cyst;
- A. Haemorrhagic cyst.
 - B. Medial palatal.
 - C. Globulomaxillary.
 - D. Nasolabial.
 - E. Radicular cyst.
86. **Dentigerous cyst is likely to cause which neoplasia?**
- A. Ameloblastoma.
 - B. Osteogenic sarcoma.
 - C. Squamous cell carcinoma.
 - D. Osteoclastoma.
 - E. Odontogenic Keratocyst.
87. **The most ideal explanation for recurrence of odontogenic keratocyst is;**
- A. Increased mitotic activity of the epithelial lining.
 - B. Friability of the epithelial lining.
 - C. Presence of satellite cysts or daughter cysts.
 - D. Continued proliferation of rests of dental lamina.
 - E. Rapid expansion of bone.
88. **Natal teeth are defined as;**
- A. Teeth present at the time of birth.
 - B. Teeth erupting in the second to third month.
 - C. Teeth erupting between 6 months to 2 year.
 - D. Teeth erupting after 1 year of age.
 - E. Teeth erupting after 2 years of age.
89. **Prognathic profile is seen in patients with;**
- A. Mandibular hypoplasia.
 - B. Skeletal Class II malocclusion.
 - C. Skeletal Class III malocclusion
 - D. Pierre-Robbins syndrome.
 - E. Stickler's syndrome.
90. **Mesiodens is a supernumerary tooth;**
- A. Between central incisor and lateral incisor.
 - B. Between central incisors.
 - C. Extra canine.
 - D. Extra pre-molar.
 - E. Extra-molar.

91. **Pierre-Robbins Syndrome is characterised by;**
A. Large mandible and cleft palate.
B. Short maxilla and cleft palate.
C. Large maxilla and cleft palate.
D. Short mandible and cleft palate.
E. Short mid-facial region and cleft palate
92. **Muscle involved in torticollis is:**
A. Temporalis.
B. Lateral pterygoid.
C. Medial pterygoid.
D. Zygomaticus major.
E. Sternocleido-mastoid.
93. **Well aligned deciduous dentition is an indication for;**
A. Potential crowding.
B. Potential spacing.
C. Disto-occlusion.
D. Mesio-occlusion.
E. Well-aligned permanent teeth.
94. **Most commonly congenitally missing tooth is;**
A .Upper lateral incisors.
B. Upper First premolar.
C. Upper central incisor.
D. Lower lateral incisor.
E. Lower canines.
95. **A 13 year old attends your clinic. She is a thumb sucker. What kind of malocclusion is she likely to have?**
A. Anterior open bite.
B. Posterior open bite.
C .Posterior cross bite.
D. Anterior cross bite.
E. Increased overbite.
96. **Which are local risk factors for Orofacial infection;**
A. Caries, impaction, pericoronitis, HIV.
B. Poor oral hygiene, periodontitis, trauma, irradiation.
C. Steroid therapy, Cytotoxics, malnutrition, anaemia.
D. Steroid therapy, Hyperthyroidism, dry socket, failed root canal therapy.
E. Malnutrition, Sick cell disease, pericoronitis, measles.

97. Ectodermal dysplasia is characterized by **ALL EXCEPT**;
- A. Generalized scarcity of body hair
 - B. Nails are dystrophic and brittle
 - C. Characterised by hypodontia and hypohydrosis
 - D. Teeth are small and tapered
 - E. Keratosis of palms of hand and soles of feet
98. A Ranula is;
- A. A large and soft mucous containing swelling on the floor of the mouth
 - B. A mucous containing pseudocyst in minor salivary gland areas of oral mucosa
 - C. An exophytic or warty mass on oral mucosa.
 - D. A benign tumour seen only in newborns
 - E. A pyogenic granuloma
99. Which is **NOT TRUE** about Burkitt's lymphoma?
- A. A type of B cell Non-Hodgkins lymphoma.
 - B. It is caused by Epstein Barr virus.
 - C. It only affects the jaws of 3-8 year old males.
 - D. The tumour mass produces rapid swelling, pain and tooth mobility.
 - E. Histopathology shows starry sky pattern.
100. Osteogenesis Imperfecta **IS NOT**;
- A. A hereditary disorder.
 - B. Characterised by multiple bone fractures.
 - C. Associated with Dentinogenesis imperfecta.
 - D. Often fatal in utero or early years of life.
 - E. Associated with normal sclerae.



MASENO UNIVERSITY

UNIVERSITY EXAMINATIONS 2017/2018

**FIFTH YEAR END YEAR EXAMINATION FOR THE DEGREE OF
BACHELOR OF MEDICINE AND BACHELOR OF SURGERY WITH
INFORMATION TECHNOLOGY**

CITY CAMPUS

SUPPLEMENTARY EXAMINATION

MCS 503: EAR, NOSE AND THROAT SURGERY

Date: 23rd May, 2018

Time: 2.00 - 5.00pm

INSTRUCTIONS:

- This paper contains One hundred (100) multiple choice questions each with options A – E.
- Choose ONE single best answer by circling the correct option
- Answer ALL Questions
- Write your admission number on ALL pages



6. Which of the following is TRUE, or likely to occur in children having severe sleep apnoea syndrome from adenotonsillar hypertrophy.
- A. Hyposomnolence
 - B. Increased appetite
 - C. Difficulties more marked in expiration
 - D. Right to left shunt in unclosed VSD
 - E. None of the above
7. Adult patients with obstructive sleep apnoea are predisposed to all of the following EXCEPT:
- A. Peptic ulceration
 - B. Hypertension
 - C. Right heart failure
 - D. Increased incidence of road traffic accidents
 - E. Weight loss
8. Which of the following organisms are NOT commonly isolated in chronic suppurative otitis media?
- A. Proteus spp.
 - B. Staphylococci
 - C. Pseudomonas spp.
 - D. Coliforms
 - E. H. influenza
9. The commonest non- epithelial tumours of the head and neck region are:
- A. Lymphomas
 - B. Sarcomas
 - C. Carcinomas
 - D. Chondrosarcomas
 - E. Adenocarcinoma
10. The most frequent cause of deafness in Kenya is:
- A. Mumps
 - B. Chronic suppurative otitis media
 - C. Malaria
 - D. Otologic trauma
 - E. Ototoxicity
11. Acoustic neuroma:
- A. Arises from the vestibular nerve
 - B. Patients present with vertigo as the first sign
 - C. Arises from the nerve axons
 - D. Majority are associated with neurofibromatosis
 - E. Epstein Barr virus is an aetiological factor

12. Which of the following is true about juvenile angiofibroma.

- A. Commonly presents in adolescent females
- B. Hormonal influences have been suspected
- C. Diagnosis is by biopsy
- D. Radiotherapy tends to control the disease hence recommended.
- E. None of the above

13. Otitis media with effusion:

- A. Is most prevalent in the second decade of life.
- B. Eustachian tube dysfunction is an important aetiological factor
- C. Is more likely in the presence of large infected tonsils
- D. Passive smoking is the major cause in developed world
- E. Bacterial infection is unlikely to play any role.

14. Foreign bodies in the nose:

- A. These are usually present in adult life
- B. Epistaxis is the commonest clinical feature
- C. Non-organic materials cause more tissue reaction than organic
- D. A bead should be removed with non-toothed dissecting forceps
- E. General anaesthetic is often required in children

15. Acute infective sinusitis:

- A. This is usually caused by a preceding coryza
- B. If secondary to dental infection, causative organism is likely to be H. Influenza
- C. Cannot be subsequent to barotrauma
- D. There is initially reduced mucosal glandular secretion
- E. An empyema is a collection of seromucinous fluid in the sinus.

16. A 60 year old presents with severe epistaxis. BP is 170/110 and haemoglobin level is 10.3g/dl. On examination there is bleeding on the anterior part of the nose and nasopharynx. After anterior nasanasla packing the blood soaks through it. You would now:

- A. Use another anterior merocel pack
- B. Pack posteriorly as well as anteriorly
- C. Give 2-3 units of fresh blood
- D. Give anti-hypertensives to see if bleeding abates
- E. Refer to ENT for ligation of the ethmoidal and internal maxillary arteries

17. A 3 year old boy presents with snoring. On examination the tonsils are enlarged but the adenoids cannot be seen. The best way to examine the adenoids is by:

- A. Nasopharyngoscopy
- B. Lateral soft tissue X-Ray of the neck and nasopharynx
- C. Tonsillectomy and adenoidectomy
- D. Finger palpation of the nasopharynx
- E. Examining the post-nasal space

- 18. A 35 year old female complains of recurrent dizziness in which the room spins about, lasting for 2 – 4 hours. There is associated hearing loss and tinnitus. The likely diagnosis is:**
- A. BPPV
 - B. Meniere's disease
 - C. Vestibular neuronitis
 - D. Basilar artery insufficiency
 - E. Drop attack
- 19. Lymphatics from metastatic carcinoma in upper aero digestive tract drains to lymph nodes in level;**
- A. I
 - B. II
 - C. III
 - D. IV
 - E. V
- 20. A 24 year old presents with a painful ulcer on the lateral part of the tongue. A 1.5 cm rubbery node is found in level 1. Your provisional diagnosis is:**
- A. Lymphoma
 - B. Syphilis
 - C. Aphthous ulcer
 - D. Squamous cell carcinoma
 - E. Geographical tongue
- 21. A 6 week old infant presents with a lump in the right lateral neck. It is firm, fusiform and lies between the mastoid process and the sternal notch. The likely diagnosis is:**
- A. Pseudo tumour of the sternocleidomastoid
 - B. Cystic hygroma
 - C. Dermoid
 - D. Thyroglossal duct cyst
 - E. Branchial sinus
- 22. A 6 year old boy presents with recurrent mild epistaxis. It is precipitated by physical activity and bumping his nose. Examination reveals a crust on the nasal septum. You would recommend:**
- A. Digital pressure for 5 minutes
 - B. Silver nitrate cautery
 - C. Thermal cautery
 - D. Nasal packing
 - E. Cold compress on forehead

23. A 54 year old dock worker presents with gradual onset hearing loss. (Audiogram shown shows hearing loss of about 40dB with a dip in the 4kHz range. You would offer him:
- A. Cochlear implant
 - B. No treatment
 - C. Hearing aid
 - D. Surgery
 - E. Topical antibiotics
24. A 19 year old college student presents to you with acute tonsillitis, occurring about 4 – 5 times a year for the past 3 years. She misses 3-4 days of school with each episode. You would recommend
- A. A 30 day course of cloxacillin
 - B. Tonsillectomy
 - C. Tonsillectomy and adenoidectomy
 - D. Flu vaccine
 - E. Uvulectomy
25. Nasopharyngeal carcinoma most frequently metastasizes to which lymph node levels?
- A. I and II
 - B. II and III
 - C. III and IV
 - D. IV and V
 - E. II and V
26. A man presents with a painless lump in the neck. FNAC results proved inconclusive. What is your next step?
- A. Perform a CT scan of the neck
 - B. Perform nasopharyngoscopy, oesophagoscopy, bronchoscopy, laryngoscopy and do blind biopsies of the nasopharynx, vallecula, pyriform fossae.
 - C. Tell the patient not to worry and discharge him
 - D. Perform open biopsy on the lump
 - E. Perform ultrasound of the neck
27. A 79 year old man has swallowed a fish bone, which has localised to the distal third of the oesophagus. The mortality rate from foreign bodies at this site is approximately:
- A. 0%
 - B. 25%
 - C. 50%
 - D. 75%
 - E. 100%

28. The most accurate test for confirming allergic rhinitis is:

- A. RAST
- B. Skin prick
- C. Intradermal food provocation test
- D. Full haemogram
- E. Peripheral blood film

29. A 30 year old housewife complains of a 3 year history of rhinorrhoea and nasal obstruction, with occasional sneezing. The most likely diagnosis is:

- A. Occupational rhinitis
- B. Perennial allergic rhinitis
- C. Food allergy
- D. Vasomotor rhinitis
- E. Atrophic rhinitis

30. A man presents with chronic nasal obstruction. Examination reveals bilateral huge polyps. You would perform/order:

- A. Biopsy one of the polyps to exclude malignancy
- B. Plain x-rays of the sinuses because it is the cheapest investigation
- C. CT scan as it demonstrates polyps best
- D. CT scan to investigate the linings of the sinuses.
- E. MRI

31. A man complains of foul smelling discharge from his ears for a few months. Otoscopy reveals white debris in the external ear canal. The most definitive treatment is:

- A. Antibiotics
- B. Myringoplasty
- C. Aural toilet
- D. Modified radical mastoidectomy
- E. Ear syringing

32. A 23 year old student presents with sore throat and pyrexia. Examination reveals 3+ tonsils, which on previous occasions, have regressed within 2-3 days of a course of oral Amoxicillin. You would prescribe:

- A. Amoxicillin 750mg TDS for 3 days
- B. Amoxicillin 750mg TDS for 5 days
- C. Amoxicillin 750mg TDS for 10 days
- D. Cephalosporins for 2 days
- E. Sulmethoprim – cotrimoxazole for 2 days

- 33. A 3 year old girl suffers from recurrent URTIs. Examination reveals a dull looking tympanic membrane with hyper-vascularity. What sequelae would you expect / tell the parents / be worried about?**
- A. Delayed speech
 - B. Obstructive sleep apnoea
 - C. Poor intellectual development.
 - D. Cor pulmonale
 - E. Hypersomnolence
- 34. A 45 year old lady complains of unilateral progressive sensorineural hearing loss over the course of 3 years. Head and neck examination is normal. The most likely diagnosis is:**
- A. Acoustic neuroma
 - B. Nasopharyngeal carcinoma
 - C. Drug ototoxicity
 - D. Noise induced deafness.
 - E. Otitis media with effusion
- 35. Hypernasal speech after repair of a cleft palate is due to:**
- A. Nasal obstruction
 - B. Result of an URTI
 - C. Failure of soft palate to reach the posterior pharyngeal wall
 - D. Nasalisation of "M, N" and "Ng" sounds
 - E. Significant adenoid hypertrophy
- 36. A 6-year old boy is brought to your office because his speech is not clear. On examination, you notice that his soft palate is short and does not move well. You would expect the patient to have:**
- A. Difficulty saying "M", "N" and "Ng"
 - B. Hyponasal speech
 - C. Difficulty saying "S", "P", "T", and "K"
 - D. Hoarse voice
 - E. No history of nasal regurgitation of liquids
- 37. You would choose to do a tracheostomy in which of the following situations:**
- A. A 2 year old infant with expiratory stridor and wheezing
 - B. A 6 month old infant with expiratory stridor and wheezing
 - C. A 6 day old new-born with cleft palate and micrognathia who has moderate cyanosis.
 - D. A 6 week old infant with a tight 2mm congenital subglottic stenosis and biphasic stridor.
 - E. A 3 year old child in the Emergency department with severe bleeding head injury after a road traffic accident.

- 38. A 28 year old woman complains of right aural drainage for the past 10 years. When she puts pressure on the ear canal with her finger she feels dizzy. This would most likely be due to:**
- A. A perforated eardrum
 - B. Infection affecting round window
 - C. Acute mastoiditis
 - D. Erosion of lateral semi-circular canal by cholesteatoma
 - E. Epidural abscess from cholesteatoma
- 39. You have seen a 10-month old infant who has had 5 episodes of fever, vomiting and red bulging tympanic membrane for the past 4 months which responds quickly to amoxicillin and in 10 days the tympanic membrane looks normal. His diagnosis would be:**
- A. Acute otitis media
 - B. Subacute otitis media
 - C. Recurrent acute otitis media
 - D. Chronic otitis media with effusion
 - E. Chronic suppurative otitis media
- 40. A 64-year old female with diet-controlled diabetes mellitus complains of a deep boring pain in her right ear for 1 week. On examination, there is a small amount of granulation tissue at the bony cartilaginous junction. The most likely diagnosis would be:**
- A. Acute localised external otitis
 - B. Acute diffused external otitis
 - C. Acute eczematous external otitis
 - D. Malignant external otitis
 - E. Otomycosis
- 41. Tonsillectomy and / or adenoidectomy**
- A. Will decrease circulatory T lymphocytes
 - B. Has no effect on immunoglobulin levels
 - C. Will decrease immunoglobulin levels
 - D. Will increase immunoglobulin levels
 - E. Will make child more susceptible to viral infections
- 42. Which of the following nerves is important in innervating the pharyngeal portion of the swallowing sensation?**
- A. Afferent fibres of glossopharyngeal nerve
 - B. Efferent fibres of glossopharyngeal nerve
 - C. Jacobsen's nerve
 - D. Afferent fibres of the superior laryngeal nerve
 - E. Efferent fibres of the superior laryngeal nerve